Adult advanced non-small-cell lung cancer (NSCLC) · ALK or ROS1 indication confirmed Adults with advanced ALK-positive or ROS1-positive NSCLC. National Cancer Control Programme (NCCP) 00243a is previously treated ALK disease; 00243b is ROS1 disease; 00243c is first-line ALK disease. Confirm the exact indication and local access; a listed indication, licence and reimbursement are different facts. This guide does not cover paediatric lymphoma or inflammatory myofibroblastic tumour.

NCCP 00243 · VERSION 7

Crizotinib monotherapy

The usual adult starting dose is 250 mg twice daily, every day, with no planned rest week. Use the strength and frequency on your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Make treatment fit into daily life

Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time.

Before starting

Confirm the exact indication, prescription, capsule formulation, baseline tests, pharmacy supply and contact numbers.

Taking treatment

Swallow 200 mg/250 mg hard capsules whole with water, with or without food; do not crush, dissolve or open them. Other formulations have different instructions. If a dose is missed, take it when remembered only if the next dose is at least 6 hours away; otherwise skip it. Never double a dose. If you vomit after a dose, do not take an extra dose; take the next dose at its usual time.

Blood tests and heart checks

Before starting: confirm the tumour test, blood count, kidney/liver tests, chest imaging, symptoms and all medicines; electrocardiogram (ECG) and electrolytes near the first dose. Liver tests are planned weekly for the first 2 months using current product information, then monthly; NCCP says every 2 weeks initially, so confirm the local plan. Blood count and kidney tests are monthly. ECG, electrolytes, pulse and blood pressure need a planned schedule and earlier review if unwell. NCCP lists monthly chest X-ray; other disease imaging follows your consultant plan.

Repeat prescriptions and interruptions

Request supplies early. If treatment is held, ask who authorises restart and confirm the new strength and frequency in writing.

Support at home

Bring a medicines list and symptom notes to reviews. Ask for help with travel, eating, work, finances and emotional wellbeing.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00243 v7 · official regimen
- EMA Xalkori product information: adult NSCLC and patient leaflet
- NCCP systemic anticancer therapy (SACT) nausea/vomiting classification V6, oral Tables 5 and 8
- NCCP NCIS Medical Oncology antiemetics V8 (2025)
- PROFILE 1007: previously treated ALK NSCLC (2013)
- PROFILE 1007 final OS: 2016 primary conference abstract
- PROFILE 1014: first-line ALK NSCLC (2014)
- PROFILE 1014 final OS (2018)
- PROFILE 1001 ROS1 update, including OS (2019)

Source reconciled; independent clinical, pharmacy and patient/PPI review pending